Study #	Study design	Dosage, route of administration and duration	Study subjects (n=number)	Mean age (Range)	Sex
C0743T09 (PHOENIX 2)	Double-Blind Placebo-Controlled	Fixed doses: Placebo (N = 410) Placebo → 45 mg SC regimen[a] (n = 197) Placebo → 90 mg SC regimen[a] (n = 195) 45 mg SC Weeks 0, 4 then q12w (n = 409) 90 mg SC Weeks 0, 4 then q12w (n = 411)	n = 1230	46.2 (18, 86)	M = 840 F = 390
C0743T12 (ACCEPT)	Assessor-Blind Active-Comparator Controlled	Fixed doses: Etanercept 50 mg (n = 347) twice weekly through Week 12 ustekinumab 45 mg (n = 209) at Week 0 and 4 ustekinumab 90 mg (n = 347) at Week 0 and 4	n = 903	45.0 (18, 81)	M = 613 F = 290

[a.] The placebo groups crossed over to receive ustekinumab (45 mg or 90 mg) at Weeks 12 and 16 then q12w

Table 16. Baseline Disease Characteristics in PHOENIX 1, PHOENIX 2 and ACCEPT

	PHOENIX 1		PHOENIX 2		ACCEPT	
	Placebo	Ustekinumab	Placebo	Ustekinumab	Etanercept	Ustekinumab
Patients randomized at Week 0	n=255	n=511	n=410	n=820	n=347	n=556
Median BSA	22.0	21.0	20.0	21.0	19.0	20.0
BSA ≥ 20%	145 (57%)	276 (54%)	217 (53%)	445 (54%)	169 (49%)	289 (52%)
Median PASI	17.80	17.4	16.90	17.60	16.8	17.1
PASI ≥ 20	91 (36%)	169 (33%)	133 (32%)	300 (37%)	102 (29%)	205 (37%)
PGA of marked or severe	112 (44%)	223 (44%)	160 (39%)	328 (40%)	148 (43%)	242 (44%)
History of psoriatic arthritis	90 (35%)	168 (33%)	105 (26%)	200 (24%)	95 (27%)	157 (28%)
Prior phototherapy	150 (59%)	342 (67%)	276 (67%)	553 (67%)	224 (65%)	368 (66%)
Prior conventional systemic therapy excluding biologics[a]	142 (56%)	282 (55%)	241 (59%)	447 (55%)	199 (57%)	311 (56%)
Prior conventional systemic or biologic therapy[a]	189 (74%)	364 (71%)	287 (70%)	536 (65%)	218 (63%)	337 (61%)

	PHOENIX 1		PHOENIX 2		ACCEPT	
	Placebo	Ustekinumab	Placebo	Ustekinumab	Etanercept	Ustekinumab
Failed to respond to, had contraindication for, or intolerant to ≥ 1 conventional therapy[a]	139 (55%)	270 (53%)	254 (62%)	490 (60%)	347 (100%)	555 (100%)
Failed to respond to, had contraindication for, or intolerant to ≥ 3 conventional therapies[a]	30 (12%)	54 (11%)	66 (16%)	134 (16%)	52 (15%)	78 (14%)

a. In PHOENIX 1 and 2, conventional systemic agents include acitretin, PUVA, methotrexate, and cyclosporine. In ACCEPT, conventional systemic agents included PUVA, methotrexate, and cyclosporine. All patients were required to be etanercept naïve at baseline in ACCEPT, but in PHOENIX 1 and 2 patients may have previously received etanercept.

PHOENIX 1 evaluated the safety and efficacy of ustekinumab versus placebo in 766 patients with plaque psoriasis. Patients were randomized in equal proportion to placebo, 45 mg or 90 mg of ustekinumab. Patients randomized to ustekinumab received 45 mg or 90 mg doses at Weeks 0 and 4 followed by the same dose every 12 weeks. Patients randomized to receive placebo at Weeks 0 and 4 crossed over to receive ustekinumab (either 45 mg or 90 mg) at Weeks 12 and 16 followed by the same dose every 12 weeks. To evaluate the efficacy of every 12-week dosing, patients who were PASI 75 responders at both Weeks 28 and 40 were re-randomized to either continue dosing of ustekinumab every 12 weeks or to placebo (ie, withdrawal of therapy). Patients withdrawn from ustekinumab at Week 40 reinitiated ustekinumab at their original dosing regimen when they experienced at least a 50% loss of their PASI improvement obtained at Week 40. Patients were followed for at least 76 weeks.

PHOENIX 2 evaluated the safety and efficacy of ustekinumab versus placebo in 1230 patients with plaque psoriasis. This study design was identical to PHOENIX 1 through Week 28.

Dose Adjustment (every 8 weeks)

At Week 28, PHOENIX 1 patients who were nonresponders (< PASI 50 response) discontinued treatment and patients who were partial responders (≥ PASI 50 response and < PASI 75 response) were adjusted to every-8-week dosing. PASI 75 responders at Week 28 who became partial responders or nonresponders at Week 40 were adjusted to every-8-week dosing.

In PHOENIX 2, patients who were partial responders at Week 28 were re-randomized to either continue every 12 weeks dosing of ustekinumab or to switch to every 8 weeks dosing.

All patients were followed for up to 76 weeks in PHOENIX 1 and up to 52 weeks in PHOENIX 2 following first administration of study treatment.

In both studies, the primary endpoint was the proportion of patients who achieved a reduction in score of at least 75% from baseline at Week 12 by the PASI (PASI 75). Patients achieving ≥ 90% improvement in PASI from baseline (PASI 90) were considered PASI 90 responders and patients with ≥ 50% improvement in PASI from baseline (PASI 50) were considered PASI 50 responders. Another key efficacy assessment was the Physician's Global Assessment (PGA), a 6-category scale ranging from 0 (cleared) to 5 (severe) that indicates the physician's overall assessment of psoriasis focusing on plaque thickness/induration, erythema, and scaling.